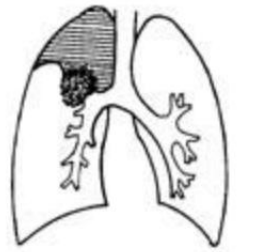
4.影像学检查（若 X 线发现病变→CT）：发现及定位

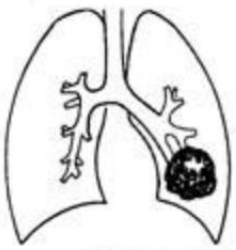
①结节或肿块伴分叶、毛刺、凹凸不平的空洞

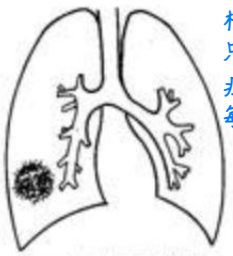
②右上叶中央型肺癌可有倒 S 征 堵塞气道→肺不张→水平裂被提上来(左边没有水平裂)

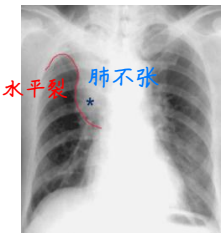
③PET/CT 发现早期肺癌和转移灶、分期、疗效评价最优 PET(代谢层面):代谢旺盛→葡萄糖高摄取→核素浓聚

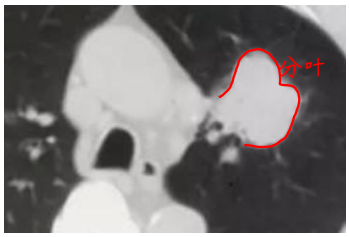
核素浓聚不代表有肿瘤，只能说明代谢旺盛(炎症时代谢也旺盛),PET敏感性高,特异性差

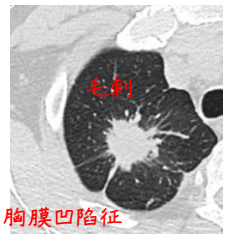

（1）右上叶中央型肺癌(肺不张)

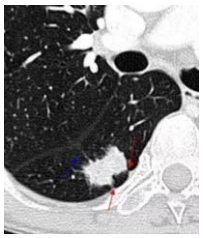

（2）左下叶癌性偏心性空洞

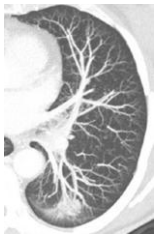

（3）右下叶周围型肺癌











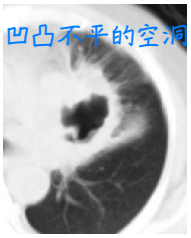
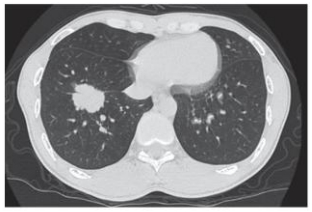



图 2-9-1　中央型肺癌
男性,60 岁。右上肺中央型肺癌并阻塞性肺不张、阻塞性肺炎。病理为肺鳞癌。


图 2-9-2　周围型肺癌
男性,52 岁。右下肺周围型肺癌并右肺门淋巴结、双肺转移。病理为肺高分化腺癌。


图 2-9-3　癌性空洞


图 2-9-5　左下肺腺癌
男性,61 岁。左下肺周围型肺癌,病理为肺腺癌。

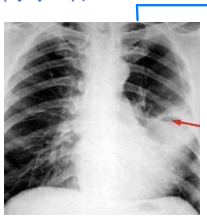

图 2-9-6　右下肺腺癌
男性,32 岁。右下肺周围型肺癌,病理为肺腺癌。

仰卧位:上叶后段或下叶背段

空洞小结:吸入性肺脓肿:炎性渗出,大片边缘不清浓密模糊浸润影,空洞内有液平

VS 支扩:薄壁囊腔,痰分4层

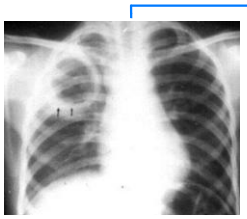
慢性肺脓肿:厚壁空洞,痰分3层

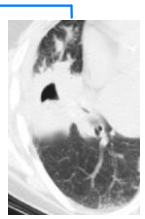


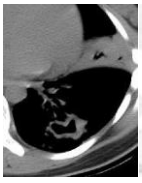


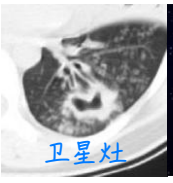








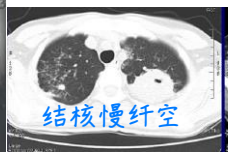


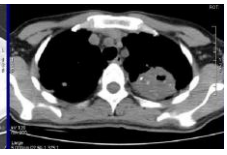












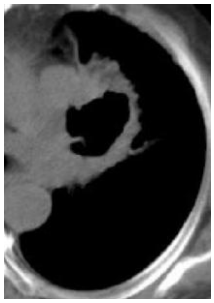
浸润性肺结核:薄壁空洞

干酪性肺炎:虫蚀样无壁空洞

慢纤空:厚壁空洞

大叶性肺炎:绝无空洞







凹凸不平:癌性空洞